MediBytes PRD

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MediBytes is an intelligence layer over a hospital's existing HMS and EMR. It captures clinical information once, checks it for contradictions, routes the resulting work to the right people, and makes sure documented care is billed. Everything below is intended behaviour, not shipped capability.

The five categories

Customers buy five intelligence products. The 16 modules sit underneath, on common infrastructure for patient records, task management and analytics.

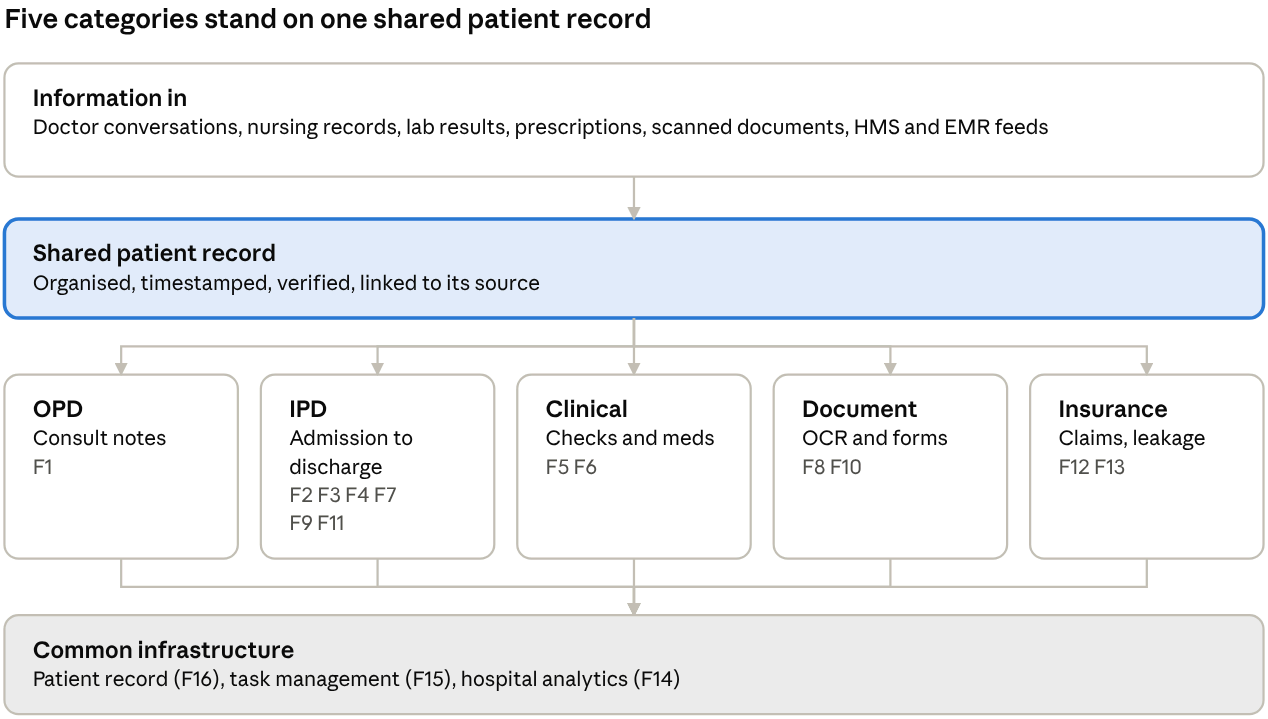
Category	What the hospital buys	Modules
OPD Intelligence	Consultation notes drafted from the doctor–patient conversation	F1
IPD Intelligence	Inpatient documentation end to end: admission, rounds, nursing, ICU, referrals, discharge	F2, F3, F4, F7, F9, F11
Clinical Intelligence	Cross-checks across diagnoses, drugs, labs, orders and plans to surface errors and omissions	F5, F6
Document Intelligence	Paper and uploads turned into structured, searchable data; forms filled from it	F8, F10
Insurance Intelligence	Claim packs prepared, incomplete claims and missed revenue detected	F12, F13
Common infrastructure	Unified patient record, task tracking, hospital analytics	F14, F15, F16

The 16 modules

#	Module	In one line
F1	Ambient OPD documentation	Listens to the consultation, with consent, and drafts the note for approval
F2	AI-assisted admission	Pre-fills the admission file from records already held; flags allergies to verify
F3	Interdisciplinary handover	Turns one verified round instruction into the note, the nursing chart and the task board
F4	Daily progress notes	Drafts today's entry from what changed since the last note
F5	Clinical Guardrail	Checks drugs, labs, contradicting notes and unresolved actions, with evidence attached
F6	Medication reconciliation	Compares home list against current plan on dose, route, frequency and indication
F7	ICU flow sheets	Pulls device readings and structured entries into the right chart, with units and timestamps
F8	Medical OCR	Reads uploaded reports into structured values; keeps the original, marks uncertain readings
F9	Discharge summary	Assembles the stay from documented events only, for doctor approval
F10	Investigation and radiology forms	Pre-fills the request with indication, diagnosis and allergies to verify
F11	Smart referrals	Sends the specialist the history and the specific question; tracks the consult
F12	Insurance and TPA documentation	Builds the claim pack, checks completeness, flags contradictions before submission
F13	Revenue-leakage detection	Compares documented services against charges and flags mismatches
F14	Hospital dashboard	Pending tasks, draft discharges, claims and discrepancies for management

#	Module	In one line
F15	Task and escalation engine	Created, assigned, acknowledged, completed — then escalates what stalls
F16	Unified patient timeline	Every event in order, with who recorded it, when and from which source

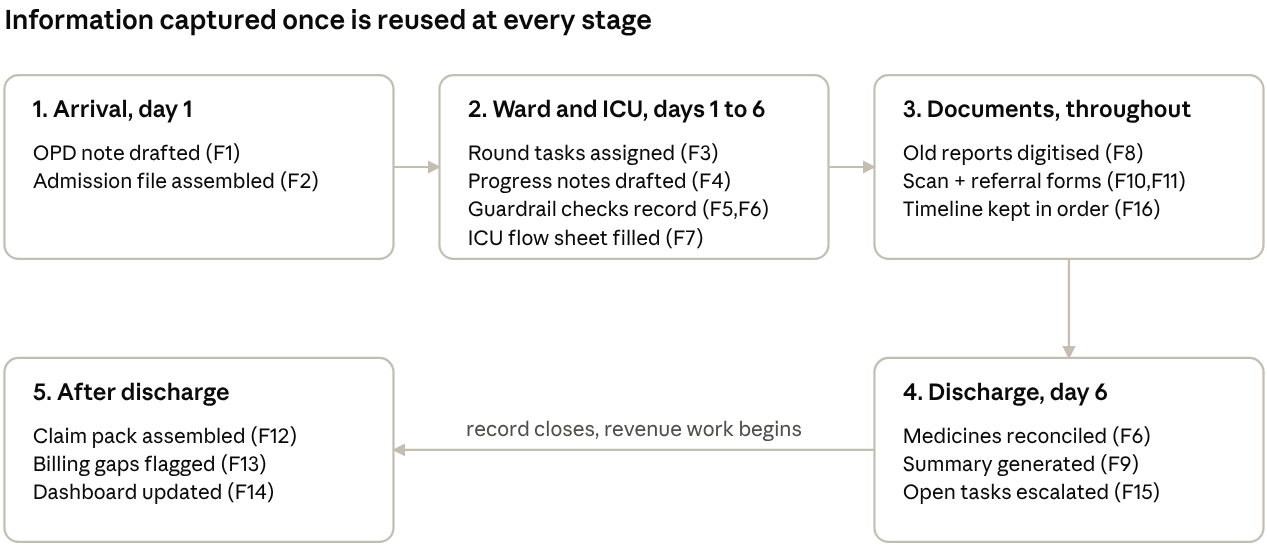
How it fits together



platform layers · inputs, shared record, five categories, infrastructure

Ravi's kidney result, entered once, is used five times: the Guardrail checks metformin against it, the progress note summarises it, the referral carries it as context, the discharge summary reports it, the timeline dates it. That reuse is the reason to build one platform rather than 16 apps.

The patient journey



patient journey · 5 stages, 16 modules

Each role sees a different product. The doctor reviews drafts instead of typing them. The nurse gets round instructions as owned tasks. The pharmacist works a queue of medication differences. Billing opens a claim pack that is already assembled. Management watches where work is stuck.

Five rules every module obeys

1. **Draft, do not decide.** Orders, medication changes and medical judgements pass through an authorised clinician.
2. **Never invent.** Only assemble what was documented — no unrecorded findings, treatments or improvements.
3. **Show the evidence.** Every alert and draft line links back to its source.
4. **Know said from ordered from done.** A discussion is not an order, a prescription is not an administration, a task is not a completion.
5. **Integrate, do not replace.** The hospital keeps its HMS and EMR; MediBytes connects by API.

Alert precision is a product requirement. Too many false alarms and clinicians ignore the system; too few alerts and they trust it wrongly.

Commercial model

A recurring platform subscription for the modules enabled, plus AI credits consumed per operation. The illustrative schedule: OCR 1, voice note 2, medicine reconciliation 2, report analysis 4, discharge summary 5, with monthly packs of 5,000, 15,000 and 50,000. These are proposed units, not final prices.

Glossary

Term	Meaning
HMS	Hospital Management System: patients, beds, billing, departments
EMR	Electronic Medical Record: the digital patient file
OCR	Reading text from scanned pages and photographs
TPA	Third-Party Administrator: administers insurance claims for an insurer
eGFR	Kidney function, mL/min/1.73 m^2; lower is worse
Reconciliation	Comparing two versions of a record and resolving the differences
Closed loop	A decision tracked from recorded to completed and accounted for